

General Paediatrics/ Paediatric Gastroenterology

Management of Constipation in Children

Take a full history and examination including:

- Frequency and consistency of stool
- Diet and fluid intake
- Behaviour including toileting
- Social history

Remember: Abdominal pain may be due to constipation and diarrhea may be overflow.

Are there any amber/red flags?

For all Children

Give general advice on fluids and diet (*see link to NICE guidance below)

Give written advice on constipation such as from

Patient info - <https://patient.info/digestive-health/constipation/constipation-in-children>

For a well, breast fed baby, passage of stools

Amber Flags

- Faltering growth
 - Disclosure or evidence that raises concerns over possibility of child maltreatment.
- Consider advice from paediatrician

Red Flags

Constipation from birth/neonatal period
Failure/delay in passing meconium > 48hrs
Ribbon stools
Weakness in legs/locomotor delay
Abdominal distension + /-vomiting
Abnormal appearance of anus (do not do a PR)
Abnormal examination of spine
Abnormal neuromuscular signs or reflexes

NO

YES

For advice from paediatrician: see Further Assistance sheet attached

Idiopathic Constipation

Reassure child and family
Start Maintenance Therapy

- Start with Macrogol (e.g. Movicol paediatric, for >12 years use standard movicol)
<1 year: ½ -1 sachet daily
1-5 years: 1 sachet daily
6-12 years: 2 sachets daily
- Re-assess frequently (within 2 weeks)
- Adjust dose to produce regular soft stool. Max 4 sachets/day as maintenance (age 1-12 years). Max 1 sachet a day for <1 years.
- If there is no effect after 2 weeks add a stimulant laxative (eg senna)

If Macrogol is not tolerated, substitute with a stimulant laxative +/- lactulose

Review all children after 4 weeks

Faecal Impaction

(suspect if LIF mass+/- soiling)

Reassure child & family Start Disimpaction Therapy

1. Start with Macrogol (e.g. Movicol paediatric, for >12 years use standard movicol)
< 1 year: ½-1 sachet daily
1-5 years: 2 sachets to start, increase by 2 sachets every 48hrs to max 8

5-11 years: 4 sachets to start, increase by 2 sachets each day to max 12
2. Review within 1 week. If there is no effect after 2 weeks add stimulant laxative (e.g. senna)
3. If Macrogol not tolerated, substitute with a stimulant laxative +/- Lactulose

Warn child and parents that disimpaction may initially increase the symptoms of soiling and abdominal pain

REVIEW overall response 4-6 weeks

Improvement

No improvement

After regular bowel habit is established **continue medication at maintenance dose** for several weeks or months.

Do not stop medication abruptly: gradually reduce the dose over a period of months.

Discuss with paediatrician or refer to general paediatrician or community clinic for further assessment
See Further Assistance sheet attached

Top Tips

- Engage and support parents (see the following useful resource: <https://www.eric.org.uk>)
- Are there non-medical factors involved?
 - o Check about toileting issues and toilet behaviour. Use reward systems such as star charts to encourage good toileting behaviour.
 - o Are they withholding because school toilets not clean etc.?
 - o Are there other emotional issues/difficulties at home?
- Do they understand the condition?
 - o Educate about constipation - Give written information.
 - o Advice about diet and fluids.
 - o Let the family know that it is a chronic condition, there is no quick fix, and treatment may be needed for months.
- Do they know how to make up and take the medication?
 - o They can mix with other drinks to make it more palatable e.g. squash.
- Don't under-medicate
 - o Do not be afraid to give high doses of medication – NICE Guidance gives higher doses than BNFC.
 - o After disimpaction the starting maintenance dose may be half the disimpaction dose.

***Table 4 from NICE Guidance: Constipation in children and young people - Laxatives: recommended doses**

Laxatives	Recommended doses
Osmotic laxatives <i>Lactulose</i>	• Child 1 month to 1 year: 2.5 ml twice daily adjusted according to response • Child 1-5 years: 2.5-10 ml twice daily, adjusted according to response (BNFC recommended dose) • Child/young person 5-17 years: 5-20 ml twice daily, adjusted according to response (BNFC recommended dose)
Stimulant laxatives <i>Senna</i> <i>Bisacodyl</i> <i>Docusate sodium</i>	Senna syrup (7.5 mg/5 ml) • Child 1 month to 2 years: 2.5-10 ml once daily – (<i>unlicensed indication</i>) • Child 2 years to 4 years: 2.5-10 ml once daily • Child/young person 4-17 years: 2.5-20 ml once daily Senna (non-proprietary) (1 tablet = 7.5 mg) not licensed in <6 years • Child/young person 6-17 years: 1-4 tablets once daily BNFC recommended doses By mouth • Child/young person 4-17 years: 5-20 mg once daily y rectum (suppository) • Child/young person 2-17 years: 5-10 mg once daily • Child 6 months-2 years: 12.5 mg three times daily (use paediatric oral solution) • Child 2-12 years: 12.5-25 mg three times daily (use paediatric oral solution) • Child/young person 12-17 years: up to 500 mg daily in divided doses

Please see the MHRA safety update on the use of Stimulant laxatives (bisacodyl, senna and sennosides, sodium picosulfate) available over-the-counter: new measures to support safe use.

https://www.gov.uk/drug-safety-update/stimulant-laxatives-bisacodyl-senna-and-sennosides-sodium-picosulfate-available-over-the-counter-new-measures-to-support-safe-use?utm_source=e-shot&utm_medium=email&utm_campaign=DSU_August2020Main1

FURTHER ASSISTANCE

This pathway has been produced by the NCL CCG to help clinicians manage the healthcare of Children and Young People. However, if you need to contact a paediatrician, please see below for contact details:

NMUH

- Consultant Paediatric hotline: Tel: 07436 283 463 (Mon - Sun 9am–9pm)
- If unanswered call the Paediatric Registrar: Tel: 020 8887 2000 bleep 195
- Advice and Guidance is through ERS

RFH (Barnet Hospital)

- Consultant Paediatric: Tel: 020 8216 4600 bleep 2902
- Paediatric Registrar: Tel: 020 8216 4600 bleep 2900
- Advice and Guidance through ERS

RFH (Hampstead):

- Consultant Paediatric: Tel: 020 3758 2000 bleep 1000 (Available 24/7)
- Urgent Referral Clinic: Tel: 020 3758 2000 bleep 1000 to discuss & confirm time for clinic (available Mon-Fri, 10 to 12am)
- Advice and Guidance is through ERS

UCLH:

- Consultant Paediatric hotline: Tel: 07803 853567 (Mon - Sun 24/7)
- If not answered call the Paediatric Registrar: Tel: 020 345 6789 bleep 5301
- Daily rapid referral clinic appointments are available through ERS (specifically choose the Children & Young People Rapid Referral Clinic, Paediatric Division-UCLH-RRV)
- Advice and Guidance is through ERS

Whittington Health:

- Consultant Paediatric hotline: Tel: 0779 694 0840 (Mon-Sun 9am - 8:30pm)
- For urgent advice on a sick child, bleep the Paediatric SpR: Tel: 020 7272 3070 bleep 3111
- 10 - 12 clinic: children seen 24 - 72 hours from referral.
Email form (on GP Portal) to whh-tr.childremsambulatorycare@nhs.net
- Advice and Guidance Email: askpaediatrics.whitthealth@nhs.net or via ERS

Additional Community Resources

Islington Paediatric Primary Care Service

<https://gps.islingtonccg.nhs.uk/service/paediatric-primary-care-team>